

Clinical Reference: Hypertension & Blood Pressure Guidelines

1. Blood Pressure Classification (ACC/AHA Guidelines)

Normal Blood Pressure: Systolic less than 120 mmHg AND Diastolic less than 80 mmHg.

Elevated Blood Pressure: Systolic 120-129 mmHg AND Diastolic less than 80 mmHg.

Stage 1 Hypertension: Systolic 130-139 mmHg OR Diastolic 80-89 mmHg.

Stage 2 Hypertension: Systolic 140 mmHg or higher OR Diastolic 90 mmHg or higher.

Hypertensive Crisis: Systolic exceeding 180 mmHg and/or Diastolic exceeding 120 mmHg.

2. Measurement Protocols and Variability

Blood pressure exhibits circadian variation and responsiveness to physical or emotional stress.

White-coat hypertension involves elevated clinic readings with normal ambulatory readings.

Diagnosis requires multiple calibrated readings across separate visits or 24-hour ambulatory monitoring.

3. Cardiovascular Risk Factors & Target Organ Assessment

Chronic hypertension contributes to atherosclerosis, left ventricular hypertrophy, and nephrosclerosis.

Diagnostic workup should include assessment of renal function (serum creatinine, eGFR), electrolytes, and urinalysis.

Primary lifestyle interventions include dietary sodium reduction (DASH diet), physical activity, and stress management.

4. Clinical Discussion Guidance

Patients with readings above 130/80 mmHg should maintain a blood pressure log.

Any abrupt systolic spike over 180 mmHg or chest pain requires emergency medical evaluation.

Medication initiation or adjustment must strictly be managed by the treating clinician.